

# NHIS DATA: SIX REAL-LIFE BENEFITS

These talking points are examples of how NHIS data have been used to benefit people in the United States. If the respondent does not engage well with a talking point, redirect the conversation to other uses of NHIS data. Remember, feel free to put these talking points in your own words.

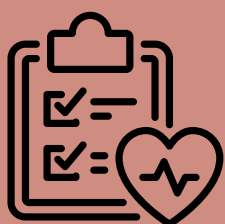
## Cancer Screenings Recommendations



NHIS data are used to update cancer screening guidelines. These help doctors know when to recommend cancer screenings that can save lives. Thanks to these updates, people are more likely to get screened at the right age and frequency to help catch cancer early.



Organizations like the American Cancer Society use NHIS data to update the guidelines that doctors use to decide when to recommend screenings that find cancer early - before it spreads, and symptoms start.



Too many of us know people who have been affected by cancer. Data from this survey are used to update cancer screening guidelines that can save lives.



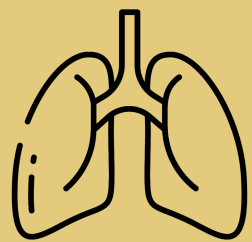
NHIS data informed the update to the recommended age to start colon cancer screening: In 2018 it changed from 50 to 45.

# Anxiety Disorder Screening Recommendations

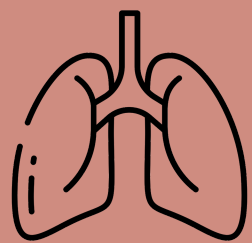


Data from NHIS informed a new recommendation that doctors screen all adults under age 65 for anxiety. This could help more people with anxiety get treatment and relief sooner.

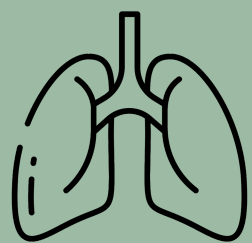
## Asthma Education and Services Program



NHIS data were used to justify the creation and continuation of programs that help many of the 24 million Americans who suffer from asthma get control of their disease and breathe easier.



NHIS asthma data have been used to show the need for programs that help millions of people with asthma avoid asthma triggers and prevent asthma attacks.



Research using NHIS data found that many kids miss school due to asthma. These findings were used to bring attention to the problem. Now there are community programs that prepare teachers to help students who are having asthma attacks.

## Diabetes Education and Services Program



NHIS data were used to inform one of the nation’s largest diabetes education programs, which helped improve Americans’ understanding and awareness of (pre)diabetes.



Program planners use NHIS data to design educational programs and materials that help people prevent diabetes and manage it if they get it, and help doctors provide better care for people with diabetes.

# Insulin Cost Reduction



REDUCE COST

Thanks in part to awareness raised by users of NHIS data, people with diabetes don't have to pay as much out-of-pocket to get insulin.



REDUCE COST

NHIS data were used to show that some people skip insulin doses or take less than they need because it cost so much. Now, insulin costs less, so the people who need it will be less likely to go without it.



REDUCE COST

Many people worry about whether they can buy insulin each month. Sometimes people skip doses or take less than they need because of high prices. Research using NHIS data were used to bring attention to this problem. Now, the country's largest producers of insulin have lowered the cost so that people who need it will be more likely to get it.

# Over the Counter Hearing Aids



NHIS data were used to show that many people who need hearing aids don't use them. Now, high quality hearing aids cost less and are easier to buy. People don't need a prescription and can buy them at pharmacies, stores, and online.



High quality hearing aids are now available over the counter, for less money, so people can buy them in stores or online without a prescription or doctor visit. NHIS data informed this change.

# Points to Consider

When using the talking points with potential respondents, consider that...

- 1

The talking points are a few examples of how NHIS data have been used, or are currently being used to benefit people in the U.S.
- 2

Researchers, community organizations, and groups like the American Diabetes Association and American Cancer Society use NHIS datasets and statistics produced by NCHS to highlight problems and design solutions. NCHS and the NHIS never make policy recommendations and do not engage in advocacy.
- 3

Since content on the NHIS rotates on and off the survey every year, topics covered in the talking points may not be asked in the NHIS in any given year.
- 4

Some talking points may not be relevant to some respondents. Use what you observe about the respondent and your judgment. For example, colorectal cancer screening may not be of interest to young adults and cervical cancer screening may not be of interest to men.
- 5

If the respondent has a negative reaction to one of these talking points, redirect the conversation back to other uses of NHIS data.
- 6

Respondents who appreciate any of the benefits mentioned in the talking points, have an opportunity to give back by participating in the NHIS. Respondents who do not appreciate the benefits have an opportunity to voice their concerns and perspectives.
- 7

By participating in the survey, respondents are also helping to ensure the availability of accurate data to power future benefits.
- 8

Remember, we encourage you to put the talking points in your own words.